

Child Marriage and Pakistan's Maternal Health Divide

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By Dr. Rubina Fareed

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Pakistan has cut the rate at which women die in childbirth by a third since 2007. Yet a woman's chance of a safe delivery still depends on where she lives, how poor her family is and how young she was when she got married. I have practised medicine for over 40 years and have served as a member of the National Commission on the Rights of Child. From both positions, I argue that Pakistan's maternal health divide is caused less by a lack of money, more by weak governance, and that child marriage lies at the heart of this problem. Many of the mothers most at risk are girls who were married before 18, and the state often meets them for the first time in a labour ward. Because our maternal health and child protection systems work separately, a pregnant child can pass through that ward without anyone recording that she is a child. I propose three practical changes: finish the reform of the marriage age, create a protected referral from maternity care to child protection, and make the health system responsible for recording births and marriages.

Context

Early in my career, I delivered two babies on the same night in the same city, perhaps 8 kilometres apart. The first mother had regular antenatal checkups. Her blood pressure and haemoglobin had been monitored for months, and a full surgical team was ready as soon as her labour became complicated. The second mother had never seen a doctor during her pregnancy. She had been in obstructed labour for more than a day, and she reached our hospital only because a neighbour owned a motorcycle and knew a doctor's phone number. We saved her that night, but only just, and I have never forgotten how close we came to losing her. Last month I saw almost the same pair of cases; the basic story has changed far less than it should have.

There has been real progress, and it should be acknowledged. Between 2007 and 2019, Pakistan's maternal mortality ratio fell by 33%, from 276 to 186 deaths for every 100,000 live births.^{1,2} A later modelled estimate, cited by the Finance Division, puts the ratio at 155 in 2023, still above the South Asian average of 120.³ One study comparing the two national surveys found that the ratio fell by 42% in rural areas, from 320 to 185, but by only 11% in urban areas, from 177 to 157, and concluded that the gap between town and country had effectively closed.¹ The official 2019 survey, however, still shows a gap: 199 deaths in rural areas against 158 in cities.² I report both figures because the sources disagree. What is clear is that the largest

differences are now between provinces. In 2019, Balochistan recorded 298 maternal deaths for every 100,000 live births and Sindh recorded 224, compared with 165 in Khyber Pakhtunkhwa and 157 in Punjab.²

Access to care shows the same pattern (Table 1). In the 2017-18 national survey, 86% of women saw a skilled health provider at least once during pregnancy, but only 51% had the recommended 4 or more antenatal visits.⁴ A skilled attendant was present at 87% of births in Islamabad and 84% in cities, but at only 63% of births in rural areas and 38% in Balochistan.⁴ Consider a woman whose high blood pressure is found at her only antenatal visit, 3 weeks before she gives birth at home without skilled help. Her problem is not biology. It is a health system that counts a single visit as care instead of following a woman through to a safe delivery.

Table 1. The risks of childbirth vary widely between provinces and between town and country.

Area	Maternal deaths per 100,000 live births, 2019	Skilled antenatal visit, 2017-18	Skilled attendant at birth, 2017-18	Birth in a health facility, 2017-18
Pakistan	186	86%	69%	66%
Urban areas	158	94%	84%	81%
Rural areas	199	82%	63%	59%
Islamabad Capital Territory	With Punjab	94%	87%	84%
Punjab	157	92%	71%	69%
Sindh	224	86%	75%	72%
Khyber Pakhtunkhwa	165	80%	67%	62%
Balochistan	298	56%	38%	35%

Source: National Institute of Population Studies and ICF, Pakistan Maternal Mortality Survey 2019² and Pakistan Demographic and Health Survey 2017-18.⁴ The 2019 survey reports Islamabad Capital Territory together with Punjab. Skilled providers are doctors, nurses, midwives and lady health visitors, and for births also community midwives. Antenatal figures refer to the most recent live birth; birth figures cover live births in the 5 years before the survey.

The second part of the divide is age. In 2018, 18% of Pakistani women aged 20 to 24 had been married before the age of 18, and 4% before 15.⁵ The rate was 34% among the poorest families but 7% among the richest.⁵ In total, some 20.5 million women living in Pakistan were married as children.⁵ In 2017-18, 8% of girls and young women aged 15 to 19 were already mothers or pregnant with their first child.⁴ Marrying young also changes the care a mother receives. An analysis of national survey data found that women married before 18 were less likely to receive antenatal care from a skilled

provider, and had more than twice the odds of giving birth at home, than women who married as adults.⁶

Young mothers also face greater dangers in childbirth. A World Health Organization study of 29 countries found that, compared with women aged 20 to 24, mothers aged 10 to 19 were at higher risk of eclampsia, infection of the womb after birth and infections that spread through the body, and their babies were more likely to be born early or underweight.⁷ Across 144 countries, the maternal mortality ratio for mothers aged 15 to 19 was about a third higher than for women in their early twenties.⁸ In Pakistan the 2019 survey recorded nearly double: 194 deaths for every 100,000 live births among mothers aged 15 to 19, against 99 among those aged 20 to 24.²

The state has little ability to see either problem. In 2017-18, the births of only 42% of children under 5 had been registered: 82% in Islamabad but 19% in Khyber Pakhtunkhwa, and 9% among the poorest fifth of families against 76% among the richest.⁴ The law, on the other hand, has changed quickly. Islamabad Capital Territory raised the minimum age of marriage to 18 for both boys and girls in 2025.⁹ Balochistan did the same in November 2025,¹⁰ and the Punjab Assembly followed in April 2026.¹¹ Sindh had already done so in 2013.¹² Khyber Pakhtunkhwa is now the main exception, as girls there may still legally marry at 16.¹³ A draft law to change this has been reviewed by the province's Law Department, but it has not yet gone to the provincial cabinet or assembly.¹⁴

Analysis

In my view, these figures point to a problem of governance more than a shortage of resources. A major review of maternal and child health in Pakistan reached the same conclusion, naming a crisis of governance and poor political ownership as the main reasons for slow progress.¹⁵ Our health workers are not the weak link. I have spent my working life among doctors, midwives and Lady Health Workers. The exhaustion I have seen in rural health centres comes from people trying to do an impossible job with too few tools, not from people who do not care. The failure lies higher up, in how the state plans, funds and, above all, checks its own health services.

The first problem is that the system values contact over continuity. The study comparing the 2007 and 2019 surveys found that, in 2019, women who died were more likely than women who survived to have received

antenatal care, and it called for better care at every stage from pregnancy through to delivery.¹ Even in hospitals, the same World Health Organization study found adolescent mothers less likely to receive standard preventive treatments, such as drugs to prevent heavy bleeding after birth.⁷ An antenatal visit that spots danger is of little use if it does not lead to a skilled birth attendant and a way to reach emergency care. Yet health programmes are judged by how many women they reach, not by whether each of those women gets safely through childbirth.

The second problem is one I see because of my previous work in the National Commission on the Rights of Child. A large share of the mothers most at risk are not women at all. They are children. The Commission is required by law to examine any law, policy or practice that affects the rights of children,¹⁶ and at the Commission we rightly discussed child marriage as a violation of a girl's rights to childhood, education and consent. In the labour ward, the same girl may lie in obstructed labour, silent because she has been taught that a wife does not question her husband's family. There she is treated only as a medical case. No one in the labour ward asks the questions the Commission would ask first: how old is this patient, and did anyone notice that she was a child before she became a patient?

This gap is built into the system. Health records note a mother's age, but her age does not trigger any action. When a girl of 15 arrives in labour, no law or procedure requires staff to refer her to child protection services, ask about her marriage or flag her case. A Lady Health Worker is trained to recognise a dangerous pregnancy, but to her a patient aged 14 is simply another clinical risk, like anaemia. In most districts she has neither the training, the resources nor the permission to treat that age as a sign that a child needs protection.

The laws on child marriage were not written with hospitals in mind either. The Child Marriage Restraint Act of 1929 never saw health facilities as a place to detect underage marriages, and the newer laws still depend mainly on nikah registrars, the police and the courts.¹⁷ Yet a health facility is one of the few places where an underage bride is almost certain to appear in an official record, because nearly every pregnancy eventually reaches the health system. Low birth registration makes matters worse. A state cannot govern what it does not count, and at present it is not counting where child marriage and dangerous pregnancies overlap.

Some will say this is simply a matter of money: more rural health centres, more community midwives and more ambulances. Money is certainly part of the answer, since public spending on health was only 0.8% of gross domestic product in 2024-25.³ But I have seen too many generously funded pilot projects quietly disappear to trust any argument that begins and ends with the budget. The deeper problem is that maternal health and child protection were set up as separate departments that have never been required to work together. Spending more money through the same separate channels would bring more women into contact with the system without giving more girls protection.

Implications

Three consequences follow. First, the national maternal mortality figure makes progress look greater than it is for the poorest women. A national average combines Islamabad with rural Balochistan, and so it hides the places where progress has been slowest. Policies judged against that average will keep rewarding gains among women who were already the easiest to reach.

Second, every underage pregnancy that reaches a health facility is a missed chance to enforce the law. The new provincial laws raise the legal age of marriage, but they are enforced mainly when someone files a complaint. Families rarely complain, and the girls involved are seldom in a position to complain. Unless a medical visit can lead to protection, these laws may remain largely on paper.

Third, the same girl is failed twice. The state fails her first when it allows the marriage, and again when she enters a health system that cannot recognise what has happened. Her child is affected too. That child is born into the kind of household where births are least likely to be registered, and so may grow up as invisible to the state as the mother.⁴

The figures in this insight come from national surveys, official records and research published in academic journals. How I interpret them, and the conclusions I draw from over four decades of clinical practice, are my own judgments and should be read as such.

The Way Forward

I am not calling for another awareness campaign. Awareness has never been the problem. The Constitution already commits the state to protect

“the marriage, the family, the mother and the child”.¹⁸ Meeting that commitment requires three practical administrative changes.

First, complete the legal reform. Sindh, Islamabad Capital Territory, Balochistan and Punjab have all set the minimum age of marriage at 18.^{9,10,11,12} Khyber Pakhtunkhwa is the main exception, even though it has had some of the highest rates of child marriage in the country.⁵ The provincial government should send the draft bill to the cabinet and the assembly without further delay. A marriage that is illegal in Islamabad should not become legal a short drive away across the provincial border. Where the new laws are already in force, provincial governments should publish yearly figures on reported cases and prosecutions.

Second, create a mandatory and protected way for maternity services to refer girls under 18 to child protection units. The aim is not to punish frightened young mothers. The Punjab law already states that children who are parties to a marriage must not be treated as offenders.¹¹ The aim is to let the state see where child marriage is concentrated and to make sure that each girl is offered protection and support. The National Commission on the Rights of Child is already required by law to inquire into violations of child rights and to keep a database on children;¹⁶ referrals would give it the evidence. The rule should be simple and written into the standard procedures of Lady Health Workers, community midwives and hospital maternity units. It must also protect confidentiality, so that no family is afraid to seek care.

Third, make the health system responsible for recording births and marriages, not only the civil registration offices. For many rural families, a health facility is the only government institution they will willingly visit. Births could be registered at the facility, or during the Lady Health Worker’s visit after delivery, and linked to the mother’s age. In South Africa, placing registration facilities in hospitals and health centres helped cut late birth registrations from more than 70% of the total in the late 1990s to less than 30% by 2010-11.¹⁹ Provincial governments and the National Commission on the Rights of Child would then have a current picture of where underage pregnancies occur.

Pakistan’s maternal health divide is not caused by geography or poverty in the way a drought is caused by the weather. It is the result of choices the state has made about which data to collect, which laws to align and which departments to make work together. Different choices can change the

outcome. A girl who arrives in obstructed labour at 14 is not first a maternal health statistic. She is a child whom the state promised, on paper, to protect. By the time she reaches the operating table, the state has already failed her twice: once when it allowed the marriage, and again when it built a health system with no way to notice it. After more than 40 years in practice, I no longer believe that the women and girls left behind will catch up if we simply wait. Closing the gap will cost less money than most people assume, but it will require more institutional honesty than we have shown so far.

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About the Author

Dr. Rubina Fareed is a physician who has practised since 1983. She has served as Member for Islamabad Capital Territory on the National Commission on the Rights of Child (NCRC), Government of Pakistan. A graduate of Khyber Medical College, University of Peshawar, she holds a Diploma in the Management of Obstetric Emergencies and Trauma from the United Kingdom and has completed certified courses in Child Protection at Harvard University and in International Women's Health and Human Rights at Stanford University. She served as Medical Director of the Melody Relief and Rehabilitation Centre after the 2005 earthquake and has chaired the caring committees of the Family Protection and Rehabilitation Centre for Women and the National Cell for the Protection of Children. Her honours include the Mohtarma Fatima Jinnah Gold Medal and the National Human Rights Award.